

GUIDELINES

S2k Guidelines (Consensus Statement) for Diagnosis and Therapy of Dermatitis Herpetiformis

Initiated by the European Academy of Dermatology and Venereology (EADV)

Abstract

Introduction

Dermatitis herpetiformis (DH) is a chronic, pruritic, gluten-induced skin disorder characterized by subepidermal granular IgA deposition and a variable degree of enteropathy identical to that seen in coeliac disease. To date, there has been no European consensus on the management of DH.

Methods

These guidelines were developed by small subgroups of a guideline committee comprising 26 specialists from various medical disciplines and one patient representative. The committee discussed and voted on the final recommendations during two consensus meetings. The process was supported by the European Academy of Dermatology and Venereology (EADV) in collaboration with the European Dermatology Forum (EDF).

Results

The guidelines provide evidence-based and expert-based recommendations (S2 level) for the diagnosis and management of DH (see Appendix).

Conclusion

These guidelines aim to improve the quality of care for patients with DH and support dermatologists in making informed diagnostic and therapeutic decisions.

Introduction

Dermatitis Herpetiformis (DH)

Definition

Dermatitis herpetiformis (DH) is a chronic, polymorphic, pruritic, gluten-induced skin disorder characterized by subepidermal granular IgA deposition and a variable degree of enteropathy identical to that observed in coeliac disease (CD). DH can therefore be considered a distinct cutaneous manifestation of CD, with combined intestinal and skin involvement.

History

The term *dermatitis herpetiformis* was first introduced in 1884 by Louis Adolphus Duhring, a dermatologist from Philadelphia, who described a chronic skin condition marked by severe pruritus and polymorphic lesions. The association between DH and systemic involvement, particularly coeliac disease, was first reported by Marks et al. in 1966, when they identified small intestinal mucosal abnormalities in DH patients. In 1967, Cormane detected immunoglobulins at the dermal-epidermal junction in DH, and two years later, van der Meer identified these as IgA. In 1979, Chorzelski and colleagues distinguished linear IgA disease from DH based on differences in direct immunofluorescence patterns. In 1983, they demonstrated that sera from patients with DH and CD contain IgA autoantibodies against endomysium. In 1990, Karpati et al. analyzed cutaneous immunoglobulins at the ultrastructural level. The key breakthrough came in 1997, when Dieterich et al. identified tissue transglutaminase (TG2) as the autoantigen targeted by anti-endomysial antibodies. Six years later, Sardy et al. identified epidermal transglutaminase (TG3) as the primary autoantigen in DH.

Genetic Background

DH is strongly associated with HLA-DQ2 or HLA-DQ8 haplotypes. Approximately 85% of Caucasian DH patients carry HLA-DQ2, with most of

the remaining 15% carrying HLA-DQ8. In Japan, a variant of DH with similar skin manifestations but without association to coeliac disease has been described. This variant is characterized by absence of gluten-sensitive enteropathy, lack of HLA-DQ2 and HLA-DQ8, frequent fibrillar IgA deposits in the papillary dermis, and no strong link to autoimmune diseases or lymphomas. Beyond genetic predisposition, environmental factors—particularly high dietary gluten intake—play a crucial role in disease development.

Epidemiology

DH is a rare condition, with a reported prevalence of 10 to 75 per 100,000 individuals and an annual incidence of 1 to 3.5 per 100,000. It is exceptionally rare in Asian populations and less common among African-Americans. Most epidemiological data derive from populations of Northern European descent, both in Europe and North America, where the disease is most prevalent. While adult onset is typical in Northern Europe, childhood onset appears more frequent in Mediterranean countries.